Machine Translated by Google

Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
PRESIDENTIAL EBOLA TASK FORCE 17



MVE17 Incident Management System

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°174/MVE-BDBV/04/11/2026

Report date: 4 November 2026 Publication date: November 5, 2026

HIGHLIGHTS (last 24 hours)

Over the past 24 hours, seventy-three (73) new confirmed cases and 32 confirmed deaths, including 23 community deaths and 9 deaths within treatment centers, have been reported. The new cases were recorded in North Kivu (36), Ituri (30), Haut-Uélé (6), and Tshopo (1). A total of 526 new samples were analyzed in the laboratories that submitted their data; 73 of these samples tested positive for the Bundibugyo virus, representing a positivity rate of 13.8%. No new health zones were affected in the past 24 hours, but the Mahagi Health Zone (Ituri) reported a new case after 91 days without a confirmed case.

ACCUMULATIVE CAS	ACCUMULATIVE DEATH CONFIRMED	LETHALITY	PATIENTS IN ISOLATION/CTE	ACCUMULATIVE HEALED	TRACKING RATES CONTACTS (Of the Day)
10741	5230	48,7%	1026	2 888	82,7%

Affected provinces and affected health zones

8 Provinces affected

Uele, Ituri, North Kivu, South Kivu, Tshopo, Lower Uele, South Ubangi, and Nord Ubangi

67 out of 178 health zones affected (37.6%)

Summary of key points from the last 24 hours

On November 4, 2026, seventy-three (73) new confirmed cases of Ebola virus disease (EVD) were reported in four provinces: North Kivu (36 cases), Ituri (30 cases), Haut-Uélé (6 cases), and Tshopo (1 case). Among these cases, 23 community deaths were reported (17 in North Kivu, 3 in Ituri, 2 in Haut-Uélé, and 1 in Tshopo). In addition, 9 confirmed deaths were recorded in Ebola Treatment Centers (ETCs) (6 in Ituri, 2 in North Kivu, and 1 in Haut-Uélé), bringing the total number of confirmed deaths reported during this period to 32.

In addition, 7 patients (4 in Ituri, 1 in North Kivu, 1 in Haut-Uélé and 1 in Tshopo) were declared cured after obtaining two negative results in the control test for Ebola virus disease.

Since the start of the epidemic, the DRC has recorded 10,741 confirmed cases, including 5,230 deaths, representing an overall case fatality rate of 48.7%. Ituri remains the epicenter of the epidemic, accounting for 70.0% of cumulative confirmed cases and 41.1% of new reported cases, while North Kivu represents 49.3% of new cases and has a case fatality rate of 54.8%, higher than the national average.

No new health zones have been affected in the last 24 hours.

Thus, 67 out of 178 health zones (37.6%), spread across eight provinces, have been affected since the start of the epidemic, namely: Ituri, North Kivu, Haut-Uélé, Tshopo, South Kivu, Bas-Uélé, South Ubangi and North Ubangi.

Machine Translated by Google

Table 1. Distribution of confirmed cases and deaths by affected province, as of November 4, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	30	7515	3 541	47,1%	28/36 (77,8 %)
North Kivu	36	2629	1 441	54,8%	17/34 (50,0 %)
Haut-Uélé	6	476	211	44,3%	7/13 (53,8 %)
Boss		95	30	31,6%	7/23 (30,4 %)
South Kivu	1	3	1	33,3%	1/34 (2,9 %)
Bas-Uélé	0	13	4	30,8%	4/11 (36,4%)
Sud Ubangi	0	2	1	50,0%	1/16 (6,3%)
Nord Ubangi	0 0	8	1	12,5%	2/11(18,2%)
Total	**73**	**10 741**	**5 230**	**48,7%**	**67/178 (37,6%)**

Table 2. Distribution of confirmed cases and deaths by province and zone as of November 4, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Cas (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	**7515**	**3541**	**47,1%**	**30**	**3**	**6**	**9**
Give	11	1	9,1%				0
Understand	25	7	28,0%				0
Ariwara	8	2	25,0%				0
Aungba	12	5	41,7%				0
Bamboo	220	50	22,7%				0
have	2	2	100,0%				0
Bunia	2025	710	35,1%	6	2	1	3
Ladies	41	10	24,4%				0
Drodro	15	7	46,7%				0
Fireworks	84	31	36,9%				0
Gethy	11	7	63,6%				0
Flounder	2	0	0,0%				0
Kilo	40	13	32,5%				0
Team	193	119	61,7%				0
Liter	333	167	50,2%				0
Logo	14	9	64,3%				0
Fight	37	11	29,7%				0
distributed	6	1	16,7%	1	0	0	0
Washing	124	65	52,4%	6	0	0	0
Darkness	515	279	54,2%	4	0	2	2
Mangala	408	174	42,6%	6	0	2	2
Mongbwalu	728	353	48,5%	2	1	0	1
Nanny	343	199	58,0%	0	0	1	1
Nizi	856	313	36,6%	1	0	0	0
Legumes	140	43	30,7%				0
Jungle	28	7	25,0%	2	0	0	0

Machine Translated by Google

Rwampara	1208	436	36,1%	2	0	0	0
Tchomia	86	27	31,4%				0
A valves	THAT	493	THAT				0
North Kivu	**2629**	**1441**	**54,8%**	**36**	**17**	**2**	**19**
Me	775	502	64,8%	16	7	0	7
Farm	26	9	34,6%				0
Butembo	346	203	58,7%	0	0	1	1
Ten	1	0	0,0%				0
Kalunguta	104	44	42,3%	1	0	0	0
Cut	683	422	61,8%	1	1	0	1
Source	3	2	66,7%				0
Kyondo	46	22	47,8%				0
Lubero	43	15	34,9%				0
Be careful.	122	39	32,0%	2	0	0	0
Alimbong	6	4	66,7%				0
Manguredjipa	16	5	31,3%				0
Masereka	47	14	29,8%	2	1	0	1
The museums	163	79	48,5%	1	1	1	2
Mutwanga	87	22	25,3%	10	6	0	6
Oicha	103	39	37,9%	3	1	0	1
Vuh	58	20	34,5%				0
Haut-Uélé	**476**	**211**	**44,3%**	**6**	**2**	**1**	**3**
Government Mangbetu	46	20	43,5%	0	0	1	1
Dumb	1	1	100,0%				0
Gombari	3	1	33,3%				0
Mathematics	119	56	47,1%	2	0	0	0
Lining	120	67	55,8%	4	2	0	2
Beans	9	5	55,6%				0
Ordinary	178	61	34,3%				0
Boss	**95**	**30**	**31,6%**	1	1	0	1
Bafwasende	18	3	16,7%				0
Kabondo	23	5	21,7%	1	1	0	1
Lubunga	1	0	0,0%				0
Maquis-Kisangani	37	14	37,8%				0
Mangobo	10	7	70,0%				0
Boss	5	0	0,0%				0
Wanie-Arugula	1	1	100,0%				0
South Kivu	**3**	**1**	**33,3%**	0	0	0	0
Miti-Murhesa	3	1	33,3%				0
Bas Uélé	**13**	**4**	**30,8%**	0	0	0	0
Blind	1	1	100,0%				0
Viadana	6	1	16,7%				0
Claim	1	0	0,0%				0
Walk	5	2	40,0%				0
Sud Ubangi	**2**	**1**	**50,0%**	0	0	0	0
Hair	2	1	50,0%				0
Nord Ubangi	**8**	**1**	**12,5%**	0	0	0	0
You are logged in.	5	1	20,0%				0
He returned.	3	0	0,0%				0
Total	**10741**	**5230**	**48,7%**	**73**	**23**	**9**	**32**

***These are confirmed deaths that occurred in the various Ebola Treatment Centers (ETCs) in the province and are currently being broken down for classification by health zone. NA: Not Applicable.**

Machine Translated by Google

Table 3. Status of alerts notified by province for November 4, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Name of suspects investigated and	Name of suspects non invest and	Name of suspects investigated and transferred At CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased			
Haut-Uélé	107	8	115	15	7	92	1	22	0	9
Bas Uélé	89	2	91	9	2	80	0	11	0	0
Ituri	1 510	87	1 597	172	86	1 338	1	258	0	128
North Kivu	1 996	66	2 062	177	64	1 819	2	241	0	141
South Bank ND		ND	ND	ND	ND	ND	ND	ND	ND	ND
North Brother ND		ND	ND	ND	ND	ND	ND	ND	ND	ND
Boss	107	0	107	7	0	100	0	7	0	2
Total General	**3 809**	**163**	**3 972**	**380**	**159**	**3 429**	**4**	**539**	**0**	**280**

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

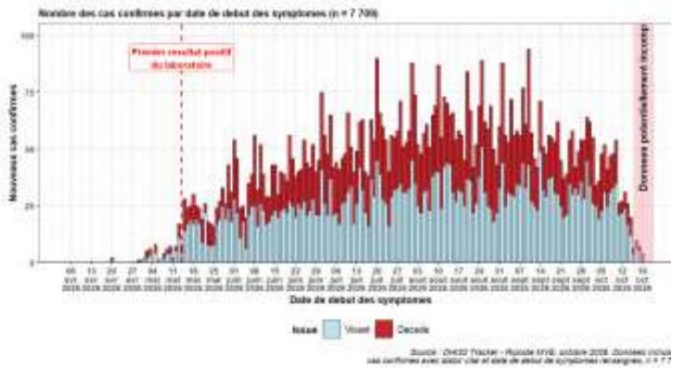
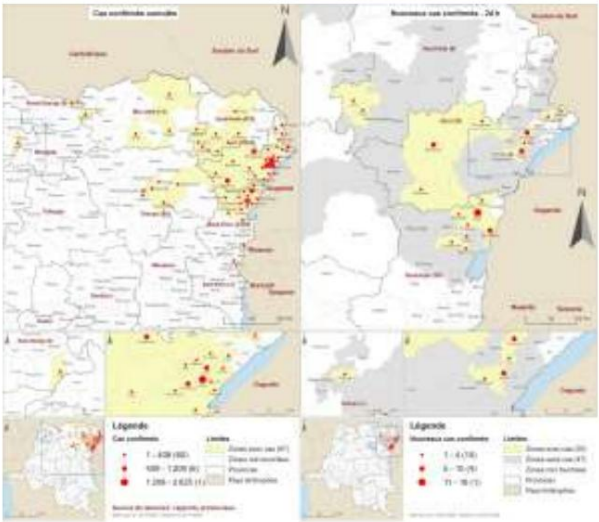


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



Machine Translated by Google

1.COORDINATION

1.1. Main Actions

• Support for response activities in the provinces **2.**
RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance
2.1.2. Main Actions

As of November 4, 2026, 3,972 alerts had been reported in the five provinces that submitted their data (Bas-Uélé, Haut-Uélé, Ituri, Nord-Kivu, and Tshopo), including 3,809 alerts of living persons and 163 of deaths; all were verified (100%). Of these, 539 (13.6%) were validated as suspected cases and 3,433 (86.4%) were invalidated. All 539 validated suspected cases were investigated (100%), and 280 were transferred to COVID-19 treatment centers (Table 3).

In total, 37,364 contacts were under follow-up in the five provinces with contacts in follow-up; 30,882 were seen in the last 24 hours, representing a follow-up rate of 82.7%, below the required threshold of 95.0% and down from the previous day (83.9%) (Table 4). By province, the follow-up rate is 97.3% in South Ubangi (109/112), 87.8% in Ituri (13,378/15,229), with none of Mangala's 659 contacts seen, 87.4% in Haut-Uélé (1,614/1,847), including 75.4% in Wamba (524/695), 78.3% in North Kivu (15,364/19,628), with 66.8% in Beni (5,495/8,231), with none of Kayna's 141 contacts or Vuhovi's 587 contacts seen, and 76.1% in Tshopo (417/548).

Table 4. Status of contact tracing by province, as of November 4, 2026

Province	Contacts under follow-up	Contacts vus	tracking rate
Ituri	15 229	13 378	87,8%
Haut-Uélé	1 847	1 614	87,4%
North Kivu	19 628	15 364	78,3%
Boss	548	417	76,1%
Bas-Uélé	0	0	THAT
Sud-Ubangi	112	109	97,3%
Nord-Ubangi	0	0	THAT
Total	**37 364**	**30 882**	**82,7%**

2.2. Update on checkpoint and point of entry (PoE/PoC) activities
2.2.1. Main actions

For the day of November 4, 2026, nine (9) alerts were notified to the PoE/PoC, including four (4) in Ituri, four (4) in Tshopo and one (1) in South Kivu: in Ituri, at the PoC Biakato Mayi (Mandima), invalidated after investigation (paralysis following a stroke), and a 38-year-old man with fever (37.7 °C) and headaches, validated and wishing to be transferred to the CTE in Beni due to the lack of a CTE in Mandima; at the PoC DGRPI Nizi, a 60-year-old woman with fever (38.2 °C), validated and transferred to the CTE; at the PoC Sans Plainte (Ariwara), the lifeless body of a 37-year-old man from Arua (Uganda), validated and having undergone an EDS; In Tshopo, four feverish travelers were isolated and investigated at the private ports of Congo Futur (2) and Bweta, and at the PK15 PoC on the Banalia road: two alerts were invalidated, and one individual chose to seek treatment in Kabondo. In South Kivu, a body was reported at the Kavimvira PoE, but no samples were taken. Thus, 1 out of 2 validated live alerts was transferred (50%), 7 out of 9 alerts were investigated within 24 hours (78%), and 1 out of 2 bodies was swabed or secured (50%) (Table 5).

Machine Translated by Google

Table 5. Status of key PoE/PoC indicators by province, as of November 4, 2026

Indicators	Ituri	North-Kivu	South Kivu	Tshopo	Lower Uele	Global
Report completeness	87% (54/62)	90% (19/21)	56% (5/9)	100% (18/18)	55% (12/22)	81,8% (108/132)
Strategic PoC/PoE	[1]	THAT	THAT	THAT	THAT	[1]
Number of past travelers	136 554	82 984	8 729	25 672	2 106	256 045
Proportion of travelers screened	96%	97%	99%	99%	97%	96%
% of travelers who washed their hands	95%	97%	99%	99%	98%	96%
% of those travelers made aware	99%	97%	100%	100%	72%	98%
Number of alerts notified	04	00	01	04	00	9
Number of validated live alerts	02	00	00	00	00	2
Number of lifeless bodies intercepted	01	00	01	00	00	2
% of suspected cases transferred to CT/CTE	50% (1/2)	0%	0%	0%	0%	50% (1/2)
% of lifeless bodies swabed/secured	100% (1/1)	0%	0% (0/1)	0%	0%	50% (1/2)
% of suspected cases investigated within 24 h	100% (4/4)	0%	0% (0/1)	75% (3/4)	0%	78% (7/9)

2.2.2. Challenges

In Bas-Uélé, the completeness is only 55% (12/22): 5 PoCs in the Viadana SZ did not work due to the strike of service providers and 5 did not report due to lack of connectivity and mobility, only 72% of travelers were made aware and coverage remains at 22 sites out of 61 (36%); in South Kivu, the completeness remains low (56%; 5/9), the antennas of the Baraka station do not report and the PoEs of Kavimvira, Kalundu Port and Baraka still do not have isolation devices; In Ituri, the completeness rate dropped to 87% (54/62), 6,061 travelers (4.4%) refused screening, with high refusal rates in Nzebi (45%), Pluto (35%), Frigo (29%), and Poipo (26%). The Points of Contact (PoCs) in the Nia-Nia health zone did not function (due to a two-month payment claim), and the Avakubi PoC remains closed due to insecurity. **2.3. Laboratory**

2.3.1. Main actions

- **Ituri:** 30 new positive results (27 living and 3 dead) out of 246 new samples analyzed (positivity: 12.2%).

- **North Kivu:** 36 new confirmed cases (19 living and 17 deaths) at the laboratories in Beni and Butembo analyzed 249 samples, of which 36 were positive (positivity: 14.5%).
- Tshopo: 1 new positive result (1 death) out of 3 samples received and analyzed (positivity: 33.3%).
- Haut-Uélé: 6 new positive results (4 living, and 2 deceased) out of 24 new samples analyzed (positivity: 25%).
- **Bas Uélé:** 9 new samples (8 bloods and 1 swab) from Dakwa were received at the Buta laboratory, of which 4 were analyzed, all came back negative, the others remain pending due to the breakdown of the generator.

2.4. Infection Prevention and Control (IPC/EDS)

2.4.1. Main actions

- **Ituri:** 22 out of 22 rings opened (100%), 39 out of 44 households decontaminated (88.6%), and 5 out of 6 health facilities (83.3%); 53 health assessments carried out out of 90 alerts, including 16 postponements (58.9%), 49 bodies swabed; the CREC facilitated the decontamination of 47 out of 47 households (100%), including 4 facilities and 1 church, and of 5 out of 5 health establishments, the completion of 76 out of 79 health assessments (96.2%), and the securing of of 14 bodies; the teams remain insufficient in Mangala and Mandima.

- **North Kivu:** 18 out of 27 rings opened (66.7%), 13 out of 21 households decontaminated (61.9%), and 17 out of 20 ESSs (85.0%); completion of the IPC-WASH training for service providers and hygienists in the Manguredjipa Health Zone (PAEV/UNICEF); 337 service providers briefed, 164 ESSs monitored and supported, and 10 IPC-WASH scorecard assessments conducted in 10 ESSs (4 to Beni, 1 in Oicha, 2 in Masereka, 1 in Manguredjipa, 1 in Kyondo and 1 in Vuhovi; average score: 46.9%); 173 followed by

Machine Translated by Google

functionality of DLM, 4 triage units installed (2 in Butembo and 2 in Masereka) and 1,735 liters of water provided to public places in Oicha; risk assessment of exposure of 8 PPL, including 4 at high risk (3 out of 6 in Oicha and 1 out of 2 in Beni), 11 schools equipped with PCI-WASH kits (3 in Beni and 8 in Mabalako), 6 schools assessed and 966 learners sensitized in Katwa; 71 EDS carried out out of 100 alerts, including 25 reports (71.0%), 69 bodies swabed following.

- **Tshopo:** 2 rings opened (Makiso-Kisangani and Kabondo) and decontamination of the Makiso General Referral Hospital and the Kamba Nga Yesu Medical Center (Kabondo) around the 95th and 94th confirmed cases, with provision of 2 replacement mattresses at the Boyoma Health Center; training and capacity building: 2nd day of training of 60 PPLs from the DHKIS on IPC before its reopening, monitoring and support of 14 out of 14 ESS in 4 health zones with 208 providers briefed on standard precautions (100%) and assessment of 9 ESS with a view to the installation of triage and isolation in Makiso and Tshopo; 2 complete EDS carried out and 2 bodies were swabed.

- **Haut-Uélé:** decontamination of one household and 1 out of 1 SSE around an infected PPL at CH Bolebole (Wamba); training and capacity building: 47 PPLs briefed in the affected health zones (18 in Isiro, 9 in Boma Mangbetu, 10 in Wamba and 10 in Pawa) on the wearing and removal of PPE, waste management and hand hygiene; 14 SSEs visited in 9 health areas, of which 7 were assessed (CS Tuluba: 40.3%; CS La Fraternité: 50.7%; CS Gombe: 12.9%; CSR Abetane: 9.7%; CS Babonde: 19.0%; CH Babonde: 48.0%; CS Bon Samaritain: 53.0%), and 5 functional triage units; 9 EDS carried out out of 10 alerts (90.0%) and 6 bodies swabed.

- **Bas-Uélé:** monitoring and support of 14 out of 14 ESS in the health zones of Buta, Koteli, Ganga and Viadana; training and capacity building: briefing of 37 out of 37 service providers and 4 out of 4 hygienists (100%) of 12 ESS on standard precautions and the operation of triage; provision of PPE and IPC inputs to the Bambesa CSR, Talangai and the Dembia and Mongbaya health centers; bio-cleaning of 3 out of 3 buildings at the Neru health center as well as the Mazianda and Akpudu health centers; the DLMs are only functional at the Neru (4/4) and Akpudu (1/1) health centers and triage only at the Viadana General Referral Hospital; 2 swabs carried out (Bondo and Buta).

2.4.2. Challenges.

- **In Bas-Uélé,** only 130 out of 632 service providers (20.6%) are trained in IPC-WASH, 16 out of 20 priority health facilities (HHFs) lack functional triage, 17 out of 20 lack water for 72 hours, none of the 20 have mattresses, the available chlorine is limited to 45 kg, no WASH partners support activities around cases, and 2 complete health data assessments (HDAs) could not be carried out. **In Haut-Uélé,** no ring could be opened in Isiro due to community resistance, only 4 out of 7 health zones (HZs) submitted their IPC data, IPC kits are lacking for HHFs, and IPC supervisors are not activated in 4 HZs (Rungu, Dungu, Boma Mangbetu, and Gombari). **In Ituri,** teams remain insufficient in Mangala and Mandima, IPC inputs are lacking in Mandima, Nia-Nia and Nizi and 22 failed samplings were recorded on bodies.

2.5. Holistic care

2.5.1. Main actions

In Ituri, 76 new admissions, all of suspected cases (including 3 admitted in advanced stages), were recorded, compared to 80 discharges, including 4 recoveries and 1 discharge without medical advice in Mangala. Thus , 341 patients remained in isolation at the end of the day (345 the previous day), representing an overall occupancy rate of 33.6%; 329 out of 337 patients (97.6%) received hot meals, all children under LNPE/SLM were cared for (8/8 at the CTE, 4/4 in daycare, and 66/66 in the community), 13 patients were evacuated, and 11 out of 34 transfers (32.4%) were organized.

- **In North Kivu,** 97 new admissions (17 confirmed and 80 suspected) were recorded against 136 discharges, including 1 recovery. In total, 595 patients (197 confirmed and 398 suspected, including 67 children under 5 years old and 27 critically ill) remain in isolation at the end of the day (634 the day before), including 210 outside CTE/CT, representing an overall occupancy rate of 108.8% (90.8% in CTE/CT; 385/424 beds); 11 out of 17 ambulances (64.7%) completed 22 trips, all successful (100%), for 28 calls received, 1,168 beneficiaries received hot meals (585 frontline providers, 200 accompanying persons, 195 suspected and 188 confirmed), 20 unstable confirmed patients received specific diets, 12 patient households received a free food distribution and the 17 separated or orphaned non-breastfed infants are on SLM (100%).

- **In Tshopo,** 2 new admissions of suspected cases (1 at the CTE Météo and 1 at the CT Kongakonga) were recorded against 1 discharge, that of a recovered patient (case n°85, ZS of Kabondo); 8 patients remain in isolation at the end of the day (7 the day before), representing an occupancy rate of 25.8%, none of whom are in serious condition; hospitalized patients and the 51 PPLs followed (100%) received hot meals, the 3 available ambulances (100%) completed 2 out of 2 trips successfully and, cumulatively, 61 confirmed cases received experimental treatment (58 on remdesivir and 3 on MBP134) and 590 people received post-exposure prophylaxis.

- **In Haut-Uélé,** 10 new admissions (3 confirmed, including 1 PPL at the Pawa CTE, and 7 suspected, including 1 PPL) were recorded against 8 discharges, including 1 recovery in Isiro, 6 non-cases and 1 death of a confirmed case at the Boma Mangbetu CTE; 35 patients (14 confirmed and 21 suspected) remain in isolation at the end of the day (33 the

Machine Translated by Google

(night), representing an occupancy rate of 25.4%, none being in serious condition; 34 patients, 44 companions and 75 PPLs received hot meals and 4 ambulance runs were carried out.

• **In Bas-Uélé,** 7 new admissions of suspects were recorded (2 in Bondo, 2 in Viadana, 1 in Bili, 1 in Likati and 1 in Poko), with no discharges, and 47 suspects remain isolated in the ESS due to the lack of a functional CTE.
In South Ubangi, no patients are hospitalized.

2.5.2. Challenges

In Ituri, the blood shortage persists, 35 patients are critical, 8 patients have not received food in Mandima, the reception capacity remains low in Mambasa, Kilo, Lolwa and Biakato and the Kasenyi CTE has still not reported due to the strike of providers (completeness: 27/28); **in North Kivu,** the overall occupancy exceeds the reception capacity (108.8%), the CTE/CTs are occupied at 90.8%, 210 patients are being treated outside CTE/CTs, 27 patients are critical, the completeness of the PEC data is only 79% (Mabalako, Mutwanga and Lubero missing) and UHT milk is lacking for children aged 6 to 23 months; **In Tshopo,** 2 confirmed escapees from Bafwasende remain missing, the CTE Météo was invaded by young people from Kabondo who forcibly retrieved the body of a deceased confirmed case, and confirmed case no. 84 has been refusing hospitalization for 8 days; **in Bas-Uélé,** the standardized CTE in Buta is still under construction, no ambulance is available, and none of the 47 isolated suspects are receiving nutritional support; **in Haut-Uélé,** only 2 out of 8 CTEs are standardized, the family of a confirmed PPL in Bolebole is opposing his transfer to the CTE in Isiro, network coverage remains weak in some health zones, and nutritional inputs (F75, F100, Plumpy'Nut) as well as the water and energy supply to the CTEs remain insufficient.

2.6. Vaccination

2.6.1. Main actions

• **In Ituri,** 130 people were vaccinated during the day (51 women, including 1 pregnant and 4 breastfeeding, and 79 men), bringing the total to 2,829 people vaccinated (1,188 women, including 28 pregnant and 84 breastfeeding, and 1,641 men), with 39 non-severe AEFI and 5 severe AEFI reported since the beginning; vaccination of PPL and research on the effectiveness of Ervebo against BDBV Ebola continue with the support of MSF. **In North Kivu,** 350 PPLs were vaccinated (150 at CS Bundji, Beni Health Zone, and 200 at Oicha General Referral Hospital), bringing the total to 1,967 vaccinated (1,114 men and 853 women, including 13 breastfeeding women) in 3 Health Zones (Beni: 1,641; Oicha: 200; Mabalako: 126) out of 7,105 expected (27.7%), with 633 doses remaining out of 2,600 received, no doses lost and 1 non-serious AEFI reported.

In Haut **-Uélé,** vaccination of people living with HIV (PLHIV) using the Ervebo vaccine has resumed in the Pawa health zone and is continuing in the Isiro health zone (Isiro General Referral Hospital and Neisu Hospital) and Wamba health zones, where the team was briefed on the mobile unit's approach before the site opened. **In Bas-Uélé,** a compassionate vaccination support team is deployed in the Bili health zone while awaiting central-level planning for Bondo and Monga, and pre-registration of PLHIV and people living with HIV continues; cumulatively, 2,000 out of 13,202 targeted individuals (15.1%) have been vaccinated in the province.

2.6.2. Challenges

In Ituri, the TPL/PPL target remains to be redefined and 5 serious AFIs have been notified cumulatively; in North Kivu, vaccination remains concentrated in the Beni health zone (83.4% of those vaccinated) and there are only 633 doses left, or 24.3% of the stock received; in Haut-Uélé, only the Pawa health zone has transmitted its pre-registration list of PPLs out of the four expected; in Bas-Uélé, the start-up of the CCUs is still not effective, the support of field actors is delayed and the Bili health zone has not yet had any vaccinated individuals (0/1,067).

2.7. Continuity of Care

2.7.1. Main actions • In North Kivu, the
baseline assessment of the Tulizeni Health Center noted the presence of a psychosocial assistant, the availability of essential nutritional inputs and MEGs and a maternity ward without any particular issues, but triage not respecting the circuit for the protection of patients and providers, an insufficient quantity of PPE and IPC inputs, poor waste management, the absence of a cold chain, low attendance at vaccination sessions since the epidemic, essential tests not available in the laboratory, patients under treatment who no longer attend the ESS and staff not trained in IPC, as well as the absence of a dashboard for monitoring disruptions, weekly supervision and a contingency plan for absenteeism.

• **In Tshopo,** 2 ESS were evaluated (CM Mbuyu Butoke II, AS of Libota, ZS of Makiso-Kisangani, and CS
Lobiko, Tshopo Health Zone): triage is effective in both facilities, which have oxytocin available, and the CS
Lobiko has a working refrigerator for vaccine storage, but no providers at the Mbuyu Community Health Center (CM Mbuyu) are trained on Ebola vaccination. Their consultation and antenatal care records are not up to date. The Lobiko Health Center has neither a working incinerator nor a garbage pit and has not submitted its MAPEPI (Major Epidemic Prevention and Control Program) to the BCZS (Central Bureau of Health Zones).

Machine Translated by Google

is not used in either structure and vaccination sessions were cancelled due to the strike; recovery plans for CM St Luc, Jehovah Jiré, Odile and La Source were developed, training of managers and providers on transfusion safety continued (J2) and the pillar participated in the feedback of the external review of the PEV.

- **In Haut-Uélé,** an assessment mission on access to care, biosecurity and community engagement in the response began in the Isiro health zone with Doctors of the World Belgium; the assessment of the Gombe health center (Boma Mangbetu) revealed a functional rainwater harvesting system but a dilapidated incinerator, insufficient latrines and almost no routine vaccination due to rumors about the Ebola vaccine; the mosquito net distribution campaign continues with the integration of Ebola messages; in Ituri, 6 health zones were supplied with essential medicines and other inputs as part of free healthcare and 28 members of the community health centers were trained for the continuity of vaccination; In Bas-Uélé, the evaluation of the functionality of the ESS in the face of the strike of non-medical health professionals shows that only 188 agents out of 515 (36.5%) are present in the 13 HGR and 84 out of 192 (43.8%) in 8 CSR, with the lowest rates of presence in Aketi (20.0%), Poko (27.1%) and Titule (27.6%).

2.7.2. Challenges

Implementation of the recovery plans for health and social care facilities (HSCs) assessed in Tshopo is still pending, where the strike by public HSC staff and the non-activation of the Expanded Program on Immunization (EPI) continuity plan persist, resulting in a decline in the use of curative services, IMCI, ANC, maternity care, and vaccination; in North Kivu, the Tulizeni Health Center lacks a cold chain, essential laboratory tests, and a contingency plan for absenteeism, and there are too few community health workers to reach pregnant women, chronically ill patients, and malnourished individuals; in Haut-Uélé, the start of the catch-up vaccination project (UIV) is delayed, the subsidy for free care is lacking in the 7 most affected health zones, and only 19 out of 67 HSCs have been assessed; in Bas-Uélé, the strike by non-physician health professionals deprives the 13 general referral hospitals (GRPs) of 63.5% of their staff; no data transmitted by South Ubangi and North Ubangi.

2.8. Risk Communication and Community Engagement (RCCE)

2.8.1. Main actions

- **In Ituri,** CREC raised awareness among 123 people, mostly students, around two deaths in the Lopa AS (Nizi), which helped to overcome resistance and carry out 2 EDS and 2 swabs, and conducted advocacy with the head of the Nyakasanza commune (Bunia) to involve community mutuals in raising alerts and accepting EDS; in support of the other pillars, it facilitated the pre-listing of 424 contacts around 9 confirmed cases, the decontamination of 47 out of 47 households (100%), including 4 structures and 1 church, and of 5 out of 5 ESS, the carrying out of 76 out of 79 EDS (96.2%) and the securing of 14 bodies, the announcement of 21 positive results out of 21 and the transfer of 39 out of 42 suspects (92.9%) to the CTE.

- **In North Kivu,** 19,412 home visits reached 54,305 people, 257 educational talks were organized for 4,661 people, 89 mass communication sessions reached 10,195 people and 6 briefings reached 387 people; 747 out of 650 expected alerts (114.9%) were notified, 26 out of 37 feedbacks (70.3%) and 7 out of 16 incidents (43.8%) were resolved within 48 hours and 2,636 out of 4,750 people (55.5%) were engaged in community actions; in Beni, the bishops, pastors and imams of the province made a solemn declaration committing to support the response and the updating of the CREC plan began; In Butembo, advocacy was carried out with the national vice-president of the association of customary chiefs and a provincial deputy; public forums brought together 60 authorities from Musienene and civil society leaders from Vuhumbani (Mabalako), and the CREC facilitated the listing of 78 contacts around the latest cases (35 in Biena, 34 in Butembo, 8 in Mabalako and 1 in Manguredjipa).

- **In Haut-Uélé,** 500 home visits reached 3,521 people, 70 educational talks reached 840 people and 6 mass activities reached 3,129 people, for a total of 7,490 people sensitized; 9 community alerts (3 in Pawa, 3 in Isiro, 2 in Boma Mangbetu and 1 in Wamba) and 3 death alerts, all resolved, were notified; 57 people, including 39 women, linked to 4 community deaths were sensitized to contact tracing, decontamination and EDS (AS of Bavemo, Tshakatsha, Mambunga and Gossamu), sensitization continued in priority schools in Isiro with the provision of thermoflashs to 3 affected schools (EP Tely 1, EP Kasongo and Collège Les Étoiles) and the CREC supervised the integration of Ebola messages into the distribution of mosquito nets in the AS of N'Sele, Gossamu and Mayogo.

- **In Tshopo,** 1,809 home visits reached 5,484 people, 4 educational talks reached 314 people, and 12 mass awareness campaigns reached 13,588 people (Lubunga, Kabondo, Makiso, Bafwasende, and Tshopo). Two programs were broadcast on RTNC and RFO. 71 community alerts were notified and resolved (41 in Makiso, including 1 death, 12 in Kabondo, 11 in Tshopo, and 7 in Mangobo). Three feedback reports were collected, and one out of every two incidents was resolved. The CREC supported contact tracing related to the 95th case and the death in Kabondo.
  obtained acceptance of 2 EDS by a family from AS De La Paix (Kabondo), overcoming a reluctance from the group

Machine Translated by Google

Kabondo pressured the EDS and held a meeting with partners around the national president of the CREC.

In Bas-Uélé, 165 home visits reached 1,033 people, 4 mass awareness campaigns reached 875 people, and 1 radio broadcast reached approximately 25,985 listeners in Buta; 172 group leaders and community leaders were sensitized in Viadana, 11 community alerts were reported and resolved (Viadana), the capacity of health workers in the health districts of Titule I, Titule II, and Malingwia was strengthened with the Red Cross on identifying and reporting alerts, and alerts from the Mopembe entry point (Titule) were monitored; only 4 out of 13 health zones reported. No CREC data was transmitted by Nord-Ubangi.

2.9. Mental Health and Psychosocial Support (MHPSS)
2.9.1. Main actions

- **In Ituri,** 13 of the 23 confirmed cases (56.5%), 58 confirmed out of 92 under follow-up, 33 new suspects out of 76 and 84 suspects out of 161 under follow-up have been taken care of, 12 households of separated and orphaned children out of 12 (100%) accompanied, 55 children followed, including 19 out of 19 in nursery with 1 new admission and 1 family reunification, 4 recovered out of 4 reintegrated and 29 out of 29 followed in community, and 41 results announced, including 13 positive; 305 accompanying persons and 72 PPLs have been supported.

- **In North Kivu,** 8 out of 8 notifying health zones (100%) were covered, 29 of the 36 new cases (80.6%), 216 confirmed cases under follow-up, 256 new suspected cases and 245 suspected cases under follow-up were taken care of, 23 out of 30 (76.7%) separated and orphaned child households were accompanied, 4 out of 4 children were followed up in daycare and 28 out of 36 (77.8%) in the community, 13 recovered patients were reintegrated and 16 out of 16 were followed up in the community, 104 results were announced, including 36 positive results, 549 people (183 men, 267 women and 99 children) benefited from psychoeducation (UNICEF support), 89 households were accompanied for public health actions and 67 PPLs were supported. In Haut-Uélé, 4 new confirmed cases, 13 confirmed and 17 suspected cases under follow-up, as well as 4 new suspected cases, were taken into care in the CTEs of Isiro, Pawa, Boma Mangbetu and Wamba, 13 results announced, including 8 positive, 14 separated children followed in community, 36 caregivers and 59 PPLs supported, 12 household members supported and 5 EDS supported.

In Tshopo, 6 out of 7 reporting health zones were covered, the last 4 confirmed cases (4/4) and the 3 confirmed cases under follow-up were treated (100%), as were the new suspected case of the day and the 2 suspected cases under follow-up. The 7 confirmed cases at the Meteorological Treatment Center (CTE Météo) received psychological support, 1 positive result was announced, 181 people (including 142 accompanying persons at the CTE Météo) benefited from psychoeducation on barrier gestures, 2 households received support, 1 person with a history of COVID-19 received support, 3 out of 3 cases of resistance were overcome, and the recovered patient being monitored in the community was seen (1/1). In Bas-Uélé, only the follow-up of a discharged recovered patient was reported for the day.

2.9.2. Challenges

2.10. In Ituri, only 15 out of 28 affected health zones (53.6%) are covered, and barely 13 of the 23 most recent confirmed cases (56.5%) have received care. Inter-pillar collaboration still needs strengthening in Mandima. In North Kivu, 7 of the 36 new cases (19.4%) did not receive support. Coverage remains low around the most recent cases in Biena, Manguredjipa, Kayna, Lubero, Masereka, Kalunguta, Mabalako, Mutwanga, and Vuhovi. Ten affected health zones lack childcare facilities, and 7 out of 30 households with separated or orphaned children remain without support. In Haut-Uélé, no childcare facilities exist in Isiro, Pawa, Wamba, and Boma Mangbetu, and community health workers lack mobility and nutritional kits for weaned infants. In Tshopo, the APS (Auxiliary Police Officers) in the Wanie-Rukula health zone are not trained, and the unit lacks communication units and motorcycles. In Bas-Uélé, the psychosocial support for the three confirmed cases is lagging behind.

2.11. Logistics
2.11.1. Main actions

- **In Ituri,** 13 patients were evacuated and 11 out of 34 transfers (32.4%) were organized by ambulances, but reporting of ambulance movements remains low; for health products and medicines (HPM), 6 health zones were supplied with MEG and other inputs as part of free care, the Mambasa health zone having requested to keep the stock intended for the health centers at the BCZ for security reasons, and the health zones received vaccines, inputs and tools for routine vaccination, while the blood stock shortage persists and IPC inputs remain insufficient in Mandima, Nia-Nia and Nizi.

- **In North Kivu,** 11 out of 17 ambulances (64.7%) were mobilized for 22 trips; supplies: delivery to the person in charge of recoveries of 27 tables and 10 cabinets offered by the WHO and provision of 11 schools with PCI-WASH kits (3 in Beni and 8 in Mabalako); for the PSM, unpacking of the donation of medicines from Humanitarian Action for Africa (antibiotics, ORS and other specialties) intended for medical care and 2,600 doses of Ervebo received in total (1,967 used, 633 remaining, none lost), with support for the deployment of vaccinators in the health districts of Mangina (Mabalako) and Bundji (Beni).

- **In Tshopo,** the PEP site was moved to the Cinquantenaire area, the Kongakonga isolation site and the CTEs have been supplied with medical inputs following stock updates, preparations for the rehabilitation of the Makiso isolation unit and the expansion of the standardized Météo CTE are continuing, and the mobility of the pillars as well as the

Machine Translated by Google

Transport and accommodation for participants in the intra-action review were provided; the 3 available ambulances were mobilized (100%) for 2 successful trips out of 2; for the PSM, the laboratory is facing a reagent shortage (MVE buffer), the failure of two modules of the Radione platform and a pre-shortage of diesel for the incinerator and the generator, and the mobility requirement for the ZS remains estimated at 115 motorcycles.

- **In Haut-Uélé,** the Isiro laboratory received a batch of medical consumables and office tools from FHI 360 and 4 ambulance runs transported 4 patients (Isiro, Wamba and Boma Mangbetu); for the PSM, nutritional inputs (F75, F100, Plumpy'Nut) are lacking in the CTEs and PCI kits are missing for the ESS, while mobility, connectivity (Starlink) and the water and energy supply of the CTEs remain to be ensured.

- In Bas-Uélé, the Bondo health zone has been supplied with EDTA tubes and PCI inputs, the packaging of PCI and PEC inputs for the health zones is in preparation, the rehabilitation of the meeting room and the offices of the DPS continues with the WHO and the construction of the CTE of the HGR of Buta reaches 50%; for the PSM, the stock of chlorine is limited to 45 kg, the laboratory has neither a fire extinguisher, nor an inverter, nor a computer, nor a freezer, the generator is broken down, no diesel is available and the 20 priority ESS lack mattresses.
  No logistical data has been transmitted by North Ubangi.

2.12. Security

2.12.1. Main actions

- **In Ituri,** the Avakubi PoC remains non-functional due to insecurity; in North Kivu, EDS teams were accompanied and secured for the burial of 3 confirmed cases at the Bundji cemetery and the threat against PoC providers on the Butembo axis persists; **in Haut-Uélé,** the security threat against PoC providers persists on the Bolebole axis (Pawa and Wamba); in Tshopo, young people from Kabondo invaded the Météo CTE to forcibly retrieve the body of a deceased confirmed case, which required the rapid intervention of the PNC, and security incidents were reported in the Wandugu AS (Kabondo); in Bas-Uélé, the pillar accompanied the PoE/PoC team, IOM and Africa CDC to the Télé site, assessed the security situation and secured the response at the zonal level.

2.13. PSEA (Prevention of Sexual Exploitation and Abuse)

2.13.1. Main actions

In Ituri, 60 people were briefed on the PSEAH in the Bunia health zone (12 APDEF agents, including 4 women, and 48 RECOs from the Bankoko, Nzere and Adventist health associations, including 26 women), 73 people were sensitized (49 vendors from the Kasegwa market in Bunia and 24 pregnant women and service providers in Rwampara) and the display of messages and the distribution of leaflets continue in the health zones of Bunia, Aru, Mongbwalu, Mambasa, Fataki and Mandima;

In North Kivu, 50 parents of students at the Ngoma Institute (30 women and 20 men) were made aware of Manguredjipa and 26 hygienists and service providers from CH Makasi, CBCA and La Lumière (19 women and 7 men) were in Butembo on the occasion of their PCI training supported by the IRC, all having signed the code of good conduct;

In Bas-Uélé, 29 learners from the ITM of Poko (13 boys and 16 girls) and 25 providers from the HGR of Poko (12 men and 13 women) were sensitized on the PRSEAH.

2.13.2. Challenges

- Coverage of affected areas remains low: only Ituri, North Kivu and Bas-Uélé reported their PSEA activities for the day (3 out of 8 DPS; 37.5%); in Bas-Uélé, 4 ZS (Viadana, Bondo, Likati and Ango) remain silent, funding for the pillar is low and posters and banners are out of stock; the reporting mechanism is known in only 15 out of 36 ZS (41.7%) in Ituri, 6 out of 13 ZS (46.2%) in Haut-Uélé and 4 out of 34 ZS (11.8%) in North Kivu, and awareness tools, visibility materials and computer kits for the pillar remain insufficient.

Machine Translated by Google

MESSAGES CLES

Seventy-three (73) new confirmed cases were reported in four provinces,
including 36 in North Kivu, 30 in Ituri, 6 in Haut-Uélé (2 in Isiro and 4 in Pawa) and 1 in the
Tshopo (Kabondo). Transmission remains particularly marked in the health zones of Beni (16 cases) and
Mutwanga (10 cases) in North Kivu and in those of Bunia, Mangala and Mambasa (6 cases each) in Ituri,
where the Mahagi health zone reports one case
after 91 days without a confirmed case.
Of the 32 confirmed deaths reported, 23 are community deaths (17 in the 19th century).
North Kivu, 3 in Ituri, 2 in Haut-Uélé, and 1 in Tshopo) and 9 are deaths at Ebola Treatment Centers (6 in
Ituri, 2 in North Kivu, and 1 in Haut-Uélé). The contact tracing rate is 82.7% (30,882/37,364) in the five
provinces with contacts currently being followed up.
(South Ubangi: 97.3%; Ituri: 87.8%; Upper Uele: 87.4%; North Kivu: 78.3%; Tshopo
: 76.1%), down from the previous day (83.9%) and still below the 95% threshold
%, with the lowest rate in Tshopo, where the health zones of Bafwasende and Kabondo did not transmit
their data due to insecurity.

TIMELINE OF KEY EVENTS

From April 24 to May 15, 2026, the epidemic progressed from
the detection of the first suspected cases to the biological
confirmation of the Ebola Bundibugyo virus.
leading to the official declaration of the 17th
Ebola Virus Disease Outbreak in the Republic
Democratic Republic of Congo.

Between May 17 and 18, 2026, the authorities
national, with the support of partners, have
strengthened the response by declaring a public health
emergency of national and then continental scope, accompanied
by the mobilization of
funds.

Machine Translated by Google

TASK FORCE PRESIDENTIAL EBOLA 17

Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

MVE17 Incident Management System

Director General of INSP

Dr. MWAMBA KAZADI Dieudonné

Email: dieudonne.mwamba@insp.cd **Incident Manager:** Professor Dr. AHUKA MUNDEKE Steve Email: steve.ahuka@unikin.ac.cd

Operations Manager

Dr. KOBBE NGWAMA John

Editorial Team

Professor Dr. MULANGU KATULUMBA Félix-Médard

Dr. KATUSI BISELENGE Jules

Mrs. WISE DIE Marie

Dr. DIUR MANANASHI Floryan

Mr. Kamuina Kebela John

Contact :

www.insp.cd

contact@sgiebola17.org

Freephone number: 151. www.sante.gouv.cd